Patient Group Direction: **Adrenaline 1:1000 Injection**			
Version: **1.0**	Date of introduction: **12/06/2025**	▼Black Triangle Product: **NO**	Classification: **POM**

For the administration only of the above medicine(s) by Registered Practitioners employed by the Trust.

This patient group direction must be agreed to and signed by registered health care professionals involved in its use. The service lead or manager is responsible for holding the original signed copy in the clinical setting where the PGD will be used. An electronic copy can be found in the approved PGD database on the Trust intranet.

Developed / Reviewed by:		
Lead Author	**Name**: Serena Skinner **Signature**:	**Position**: Lead Systemic Anti-Cancer Therapy (SACT) Nurse **Date**: 04/02/2025
Physician/Consultant*	**Name**: Peter Stephens **Signature**:	**Position**: Consultant Medical Oncologist **Date**: 11/06/2025
Lead Nurse / Allied Health Professional/ Lead Practitioner	**Name**: Clare Rowley **Signature**:	**Position**: Clinical Matron **Date**: 20/03/2025
Pharmacist*	**Name**: Stephanie Mead **Signature**:	**Position**: Specialist Pharmacist Cancer Services **Date**: 14/03/2025
ALL signature boxes above MUST be complete before submission to Medicine Management Group for approval.		
Organisational authorisation by MMG (Chair, or Deputy)	**Name**: Joe Maguire **Signature**:	**Position**: Deputy Chief Pharmacist **Date**: **12/06/2025** **PGD Expiry Date: Jun 2028** (no longer than 3 years)

*For PGDs covering systemic antibiotic use, a microbiologist and/or antimicrobial pharmacist must be included in its development.

Version	Summary of Changes
1.0 (Replaces n/a)	• New document. Created/owned by cancer services team but removed speciality specifics so more generic and can be used in other areas.

You must be authorised by name under the current version of this PGD before you attempt to work according to it.

1. Clinical Condition

Indication	Emergency treatment of acute anaphylaxis. SEND A STAFF MEMBER TO CALL 2222 (or 999 if off NDDH or RD&E Hospital sites) AND STATE THERE IS A CASE OF SUSPECTED ANAPHYLAXIS.
Inclusion criteria	Patients meeting the criteria as defined in the Resuscitation Council Guidelines / Trust policy for management of anaphylaxis. Anaphylaxis is likely when all of the following three criteria are met: • Sudden onset and rapid progression of symptoms. • Life-threatening airway and/or breathing and/or circulation problems. • Skin and/or mucosal changes (flushing, urticarial, angioedema). Remember: • Skin or mucosal changes alone are NOT a sign of an anaphylactic reaction. • Skin and mucosal changes can be subtle or absent in up to 20% of reactions (some patients only have a decrease in blood pressure i.e. a circulation problem). • There can also be gastrointestinal symptoms (e.g. vomiting, abdominal pain, incontinence) Most reactions will occur over several minutes but rarely may be slower in onset.
Exclusion criteria	Contraindications: none. Children under 12 years of age.
Cautions / Need for further advice	In severe, genuine anaphylaxis there are no relevant cautions or contraindications Anaphylactic reactions should be distinguished from syncope and panic attacks.
Action if excluded	Discuss URGENTLY with a doctor if possible document 'Informed refusal' patient notes. Inform relevant healthcare professionals if patient is transferred for further management.
Action if patient declines treatment/refuses medication	• Document informed refusal in patient's records

2. Description of Treatment

Method or route of administration	Adrenaline 1:1000 (1mg/1mL) injection as **pre-filled syringe** (use 0.5mg / 0.5ml per dose) or Adrenaline 1:1000 (1mg/mL) **ampoule** (use 0.5mg / 0.5mL per dose). May be administered as an intramuscular (IM) injection preferably to the outer thigh or upper arm.
Dose	Adults and children over 12 years: 0.5mg = 0.5mL As intramuscular injection (IM).
Frequency	Single dose.
Duration of treatment	A second dose may be given after 5 minutes if there is no clinical improvement. Maximum TWO doses only.
Quantity to supply / administer	Administration ONLY. • 0.5ml of 1:1000 (1mg/1mL) ampoule or pre-filled syringe - discard remainder. • Use a second ampoule or pre-filled syringe if a further dose is necessary.
Storage	Ampoules and pre-filled syringes: • Keep container in outer carton/aluminium pouch to protect from light. • Do not store above 25°C. • Do not freeze.

3. Patient Information

Advice and information to patient/carer, including side effects	• Unusual or life-threatening reactions require immediate medical attention.
	• Side effects include: palpitations, tachycardia, sweating, nausea, vomiting, dyspnoea, anxiety, tremor and cold extremities.
	• Before discharge, a healthcare professional with the appropriate skills and competencies should offer patients (or, as appropriate, their parent and/or carer) the following:
	  ○ Explain current course of action & need for urgent medical attention.
	  ○ Information about anaphylaxis, including the signs and symptoms of an anaphylactic reaction.

	○ Be aware that up to 20% of people with anaphylaxis have biphasic (two-phase) or prolonged anaphylaxis, i.e. a reaction that resolves and then recurs later without further exposure to the trigger. The late phase reaction usually occurs within eight hours, but may occur up to 72 hours after the initial symptoms. ○ A person with protracted anaphylaxis has signs and symptoms that persist for hours or even days despite treatment, although this is rare. Patients experiencing biphasic anaphylaxis should contact emergency services without delay. • Advice about how to avoid the suspected trigger (if known).
Follow-up advice to be given to patient or carer	• All patients must be referred for urgent hospital treatment even if they appear to have made a full recovery. • Allergies and suspected adverse drug reactions should be clearly documented in the patient's notes and the GP/other relevant healthcare professionals informed. • Patient should be reviewed by their Consultant with respect to ongoing management. • Any serious adverse drug reaction should be reported to the MHRA using the Yellow Card Scheme (www.mhra.gov.uk).

4. Staff Characteristics

Registered Profession (tick those that apply)		
	☐ Chiropodists & Podiatrists	☐ Orthoptists
	☐ Dental hygienists^	☐ Orthotists & Prosthetists
	☐ Dental therapists^	☐ Paramedics
	☐ Dieticians^	☐ Pharmacists
	☐ Midwives	☐ Physiotherapists
	☑ Nurses*	☐ Radiographers
^ Cannot administer or supply controlled drugs (schedule 1-5)	☐ Occupational Therapists	☐ Speech & Language Therapists^
* Nursing Associates not eligible to use PGDs	☐ Optometrists	☐ Registered Pharmacy Technicians^

Additional Qualifications	Nil in addition
Specialist Competencies	• Completion of Trust mandatory Anaphylaxis training annually • Completion of Basic or Immediate Life Support course annually
Additional Requirements	• Evidence of CPD, and any training/competences relevant to the PGD. • Working knowledge of organisational policies (including Medicines Management Policy), Anaphylaxis Guidance

	(resus.org.uk) & Safeguarding Policies and related mandatory training.

5. Record keeping, Referral Arrangements and Audit Trail

Specify method of recording supply /administration including audit trail	The following will be recorded in the patient's health records: • Diagnosis and treatment • The dose administered and/or the quantity supplied. • Route and site of administration (if applicable) • Date and time of administration • Signature and name of person administering the medication
6. References used in the development of this PGD:	• BNF (British National Formulary) online accessed - 04 / 02 / 2025 • Summary of Product Characteristics online accessed -04/02/2025 • Emergency Treatment of Anaphylaxis: Guideline for healthcare providers. Working Group of Resuscitation Council UK (2021).

Contacts for more information on the status of this document	
Royal Devon PGDs Pharmacy Administrative Office Pharmacy Department RD&E Hospital (Wonford) Tel: 01392 402441	
PGD Template Version: ***2025-1***	*Template approved by MMG (March 2025)*

PGD Record Keeping
Expired PGDs including staff signed authorisation sheet(s) should be scanned and retained on a shared dept/service Trust network drive for a period after the expiry date as follows: - 25 years if it relates to children - 8 years if it relates to adults only - 10 years if relates to use of an implant

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PGD's DO NOT REMOVE INHERENT PROFESSIONAL OBLIGATIONS OR ACCOUNTABILITY

It is the responsibility of each professional to practice only within the bounds of their own competence and in accordance with their own Code of Professional Conduct.

Note to authorising Managers: A paper copy of the full PGD must be kept in the relevant clinical area together with this sign-off sheet. Authorised staff can be signposted to the intranet where they can view the document.

CLINICAL AREA	LOCATION / DEPARTMENT

I have read and understood the Patient Group Direction and agree to supply / administer this medicine only in accordance with this PGD.

Name of Registered Professional	Signature	Authorising Manager	Date
			DD / MM / YYYY
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